

GuideLiner V3 & TrapLiner Catheters

Interventional Product Playbook

COLLEGE OF INTERVENTIONAL TRAINING

Teleflex

Table of Contents

Introduction 2
Clinical Overview - GuideLiner V3 Catheter 2
Introduction to GuideLiner and GuideLiner V3 Catheter 6
GuideLiner V3 Catheter Device Deployment and Handling Steps 13
GuideLiner V3 Catheter Competitive Overview 16
Introduction to TrapLiner Catheter 18
TrapLiner Catheter Overview 21
TrapLiner Catheter Deployment and Handling Steps 26
TrapLiner Catheter Competitive Overview 29
Effective Targeting 32
Applying the Balanced Seller Process 33
Handling Objections and using CPR 40
The Balanced Seller Process: YOU TRY IT 44
Selling Tools and Resources 51
Marketing Materials 53
Legal and Regulatory Disclosures 54

Introduction

Welcome to the GuideLiner V3 Catheter and TrapLiner Catheter product playbook! This internal resource is intended to provide specific product and clinical information combined with resources for applying the Balanced Seller process to your everyday sales calls involving the GuideLiner V3 Catheter and TrapLiner Catheter products.

Clinical Overview - GuideLiner V3 Catheter

What is Guide Extension?

Guide extension technology was developed in response to a need for a method of increasing guide catheter backup support, improving guide stability (the ability to hold a steady position at the coronary ostium) and promoting atraumatic deep-positioning during percutaneous coronary interventions (PCI).

Backup Support, Guide Stability and Deep-Positioning

Prior to the launch of the GuideLiner Catheter in 2009, the simplest method for increasing backup support in challenging interventions was to increase the diameter of the guide catheter. An 8F guide catheter would generally provide more backup support than a 7F or 6F guide catheter with an identical curve shape.

Use of larger guide catheters, however, increase the risk of bleeding, increase the amount of contrast use, and are poorly suited for radial access procedures because the guide catheter is too large[1].

Additionally, operators may change the guide catheter tip shape from a standard Judkins to various enhanced shapes (Amplatz, Multipurpose, etc.) for additional backup support. While supportive, these enhanced backup shapes are more difficult to use and may cause coronary dissection.

Guide stability prior to the launch of the GuideLiner Catheter, when unsatisfactory, could only be overcome by changing the guide catheter to a new shape altogether or switching the access site (typically from radial to femoral).

Deep-positioning, when necessary and prior to the launch of the GuideLiner Catheter, required “active” torqueing and advancement of the guide catheter itself, which can lead to the guide catheter kinking and/or coronary dissection.

1.Feldman, DN., Swaminathan, RV., Kaltenback, LA.et al. Adoption of radial access and comparison of outcomes to femoral access in percutaneous coronary intervention: an updated report from the national cardiovascular data registry (2007-2012).

Mother-and-Child Technique

Guide extension evolved from an earlier method referred to as the "mother-and-child" technique, which involved inserting a smaller guide catheter inside a larger guide catheter. While this method worked to enable deep-positioning, there were multiple problems associated with the technique and it is no longer utilised today.

Present Day

Today, the majority of coronary interventions are performed through a 6F guide catheter via a 6F introducer sheath. Moreover, the radial artery, a vessel that easily allows 6F equipment in most adults, is becoming increasingly popular due to fewer bleeding complications and quicker time to ambulation[1]. For ergonomics, the right radial artery is generally preferred.

The rise of radial PCI has highlighted the need for, and accelerated the adoption of, guide extension catheters such as the GuideLiner V3 Catheter.

First, radial access is generally limited to 6F guide catheters unless a sheathless technique is used.

Second, approaching the heart from the right radial position often creates challenges for guide support and stability. These limitations are all largely overcome by the use of guide extension technology[1].

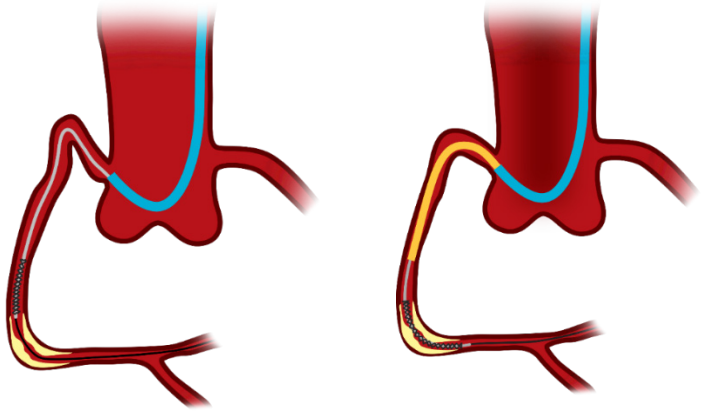
1.Feldman, DN., Swaminathan, RV., Kaltenback, LA.et al. Adoption of radial access and comparison of outcomes to femoral access in percutaneous coronary intervention: an updated report from the national cardiovascular data registry (2007-2012).

Launch of GuideLiner Catheter

In 2009, the GuideLiner Catheter was launched and offered a rapid exchange platform that delivered on the following objectives:

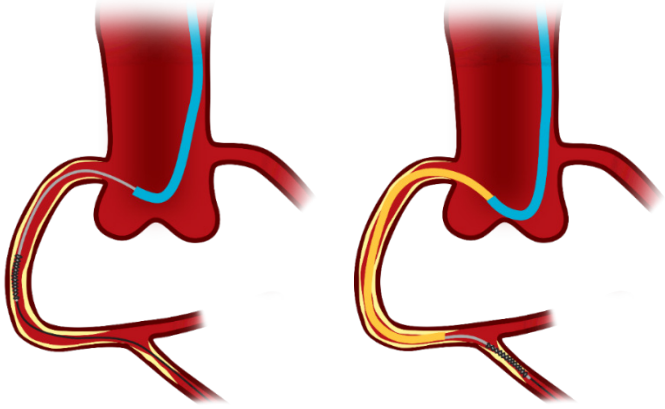
Coaxial alignment and backup support:

- The guide extension segment establishes coaxial ostial alignment necessary for devices to track smoothly into the coronary artery while putting the guide into a more supporting position against the aortic valve and opposite wall of the aorta.



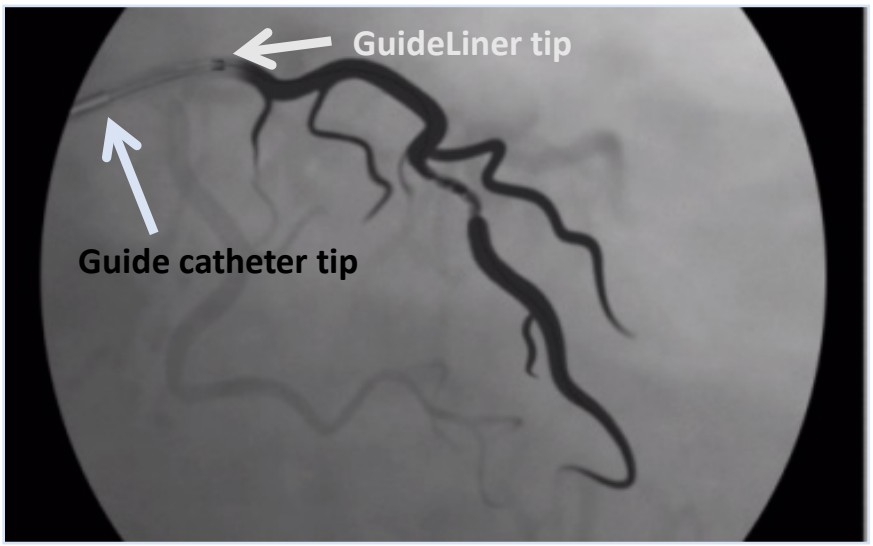
Facilitating distal device delivery:

- The guide extension deep-seats within a target vessel to facilitate distal device delivery.



Selective contrast delivery:

- The ability to deep-seat beyond or into the main vessel branches enables sub-selective injection of reduced amounts of contrast.



Summary of Guide Extension Clinical Applications Within PCI:

- Coaxial alignment and backup support
- Facilitated distal device delivery
- Selective (reduced) delivery of contrast

Introduction to GuideLiner and GuideLiner V3 Catheter

GuideLiner Catheter

The GuideLiner Catheter revolutionised the concept of guide extension in 2009 and created new possibilities in interventional cardiology. Now in its third generation, the GuideLiner V3 Catheter continues to build on a history of innovation and performance – one that has been demonstrated in cath labs around the world.

GuideLiner V3 Catheter

The GuideLiner V3 Catheter offers rapid exchange convenience that allows for the utilisation of short wires during PCI procedures. The highly flexible, coil-reinforced guide extension construction enables coaxial alignment and backup support, the ability to deep-seat within a vessel for distal device delivery, and selective (reduced) delivery of contrast.

Indications and Contraindications

- GuideLiner catheters are intended to be used in conjunction with guide catheters to access discrete regions of the coronary and/or peripheral vasculature, and to facilitate placement of interventional devices.
- GuideLiner catheters are contraindicated in vessels less than 2.5mm in diameter and in vessels in the neurovasculature or the venous system.

Features & Benefits*

The GuideLiner V3 Catheter has the following unique features:

- Coil-reinforced guide extension
- Intentional silicone-coated guide extension
- Unique half-pipe channel design
- Stainless steel, flat ribbon wire pushrod
- Broad range of sizes available

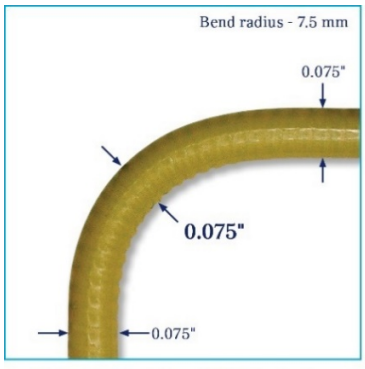
**Features & benefits are expanded upon in further detail on the next page.*

Features & Benefits (continued)

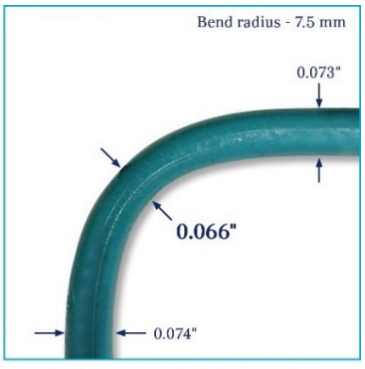
Feature: Coil-reinforced guide extension

Benefits:

- Offers greater longitudinal flexibility
    - From tab to tip, the Guidezilla II Catheter is 25% stiffer than the GuideLiner V3 Catheter[2]
- Is less kinkable than the Guidezilla II Catheter's braid-reinforced design
    - The 6F Guidezilla II Catheter is 86% more kinkable than the 6F GuideLiner V3 Catheter[2]
    - The 8F Guidezilla II Catheter is 80% more kinkable than the 8F GuideLiner V3 Catheter[2]
- Exhibits superior shape retention in tortuosity or acute bends
    - In a test comparing 7F versions of the GuideLiner V3 Catheter and the Guidezilla II Catheter, the coil-reinforced design of the GuideLiner V3 Catheter provided superior shape retention when confronted with acute bends[2]
        - As shown in the image below, the O.D. (outer diameter) of the GuideLiner V3 Catheter remained unchanged at a bend radius of 7.5 mm. The O.D. of the Guidezilla II Catheter reduced in diameter.



7 Fr. GuideLiner® V3 Catheter



7 Fr. Guidezilla™ II Catheter

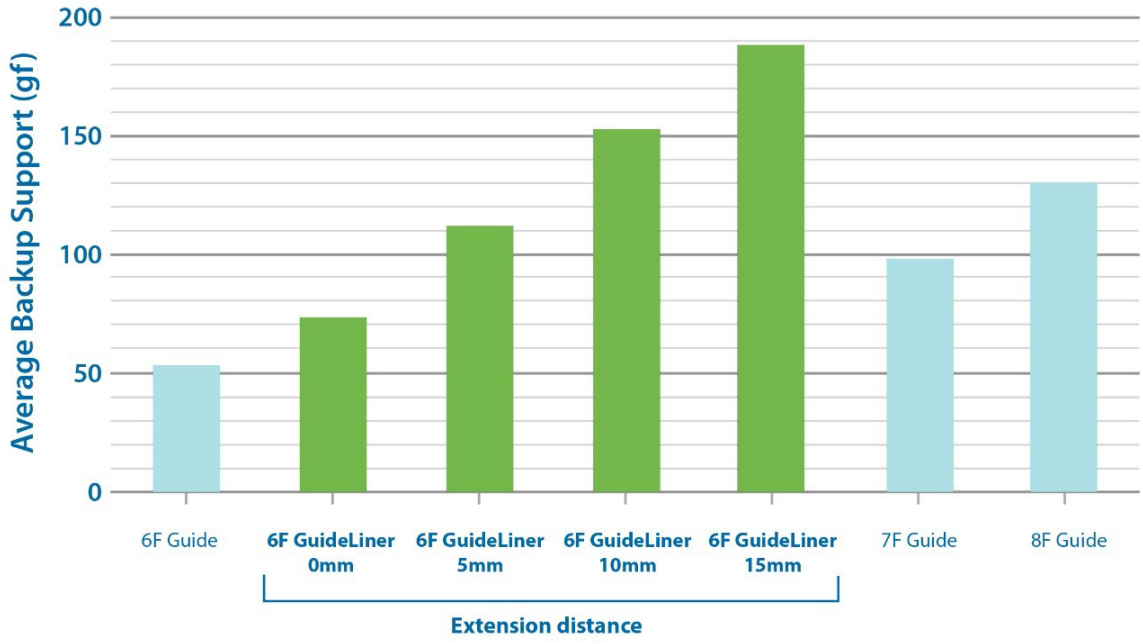
2. All values and relative product comparisons are based on bench test data averages (n≥5) of competitive models and are analysed with a minimum of 95% confidence as a percentage of the means. Bench test results may not necessarily be indicative of clinical performance. Testing completed by Teleflex. Data on file.

Features & Benefits (continued)

Feature: Silicon-coated guide extension

Benefits:

- Provides greater backup support than a fully hydrophilic-coated guide extension.
  - With as little as 5 mm of extension, bench testing has shown the GuideLiner V3 Catheter significantly increases the backup support of a 6F guide catheter[3].
  - Testing also shows the GuideLiner V3 Catheter and its silicone-coated extension provides 18% greater backup support than the Guidezilla II Catheter and its fully hydrophilic-coated extension[4]



3. Testing completed by Teleflex. Data on file. Comparative data may not be indicative of clinical performance.
4. All values based on bench test data averages (n≥5) of 6F. models and are analysed with a minimum of 99% confidence as a percentage of the means. Bench test results may not necessarily be indicative of clinical performance. Testing completed by Teleflex. Data on file.

Features & Benefits (continued)

Feature: Unique half-pipe channel design

Benefits:

- Designed to minimise device/collar interaction by directing and aligning devices through the collar transition, facilitating smooth device entry and seamless delivery.
- Creates a 17-centimeter polymer bond between the extension and the pushrod, an innovative feature designed to minimise device separation
    - There currently are zero reported collar separations in PCI for the GuideLiner V3 Catheter[5]



5. Based upon a review of all device experience reports for coronary usage of GuideLiner V3 Catheters from launch through April 30, 2019. Data on file.

Features & Benefits (continued)

Feature: Stainless steel, flat ribbon wire pushrod

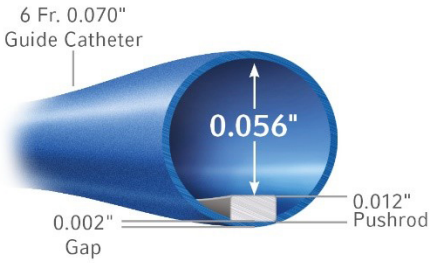
Benefits:

- Occupies less space than the circular pushrod utilised in the Guidezilla II Catheter
- Provides a larger effective I.D.* (inner diameter) within compatible guide catheters

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension segment.

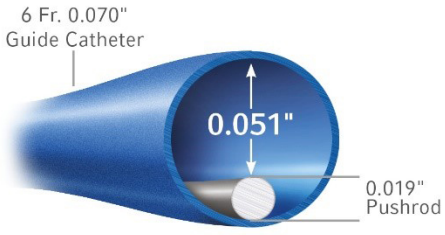
6 Fr. GuideLiner Catheter

0.070" I.D. 6 Fr. Guide
Pushrod = 0.012" (h) + 0.002" (gap)
Effective I.D. = 0.056"



6 Fr. Guidezilla II Catheter

0.070" I.D. 6 Fr. Guide
Pushrod = 0.019"
Effective I.D. = 0.051"



Features & Benefits (continued)

Feature: Broad range of sizes available

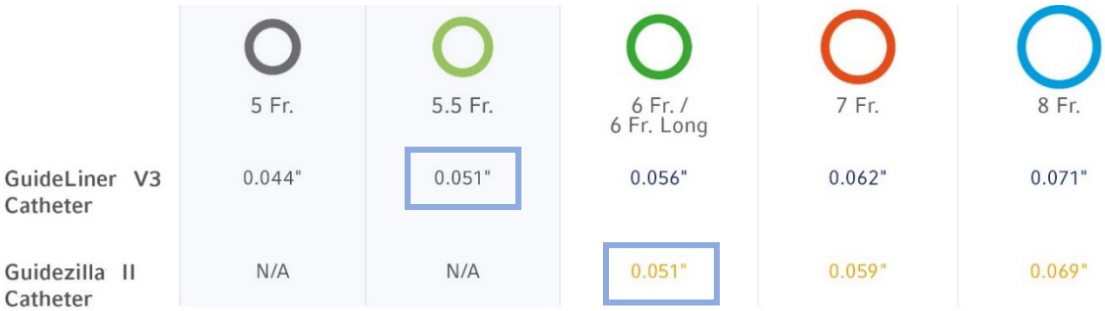
- 5F → for use through a 5F (≥0.056" I.D.) guide catheter
- 5.5F → for use through a 6F (≥ 0.066" I.D.) guide catheter
- 6F → for use through a 6F (≥ 0.070" I.D.) guide catheter
- 7F →for use through a 7F (≥ 0.078" I.D.) guide catheter
- 8F → for use through an 8F (≥ 0.088" I.D.) guide catheter

Benefits:

- The GuideLiner V3 Catheter product offering includes 5 and 5.5F sizes, beyond the 6, 7 and 8F sizes that are currently offered in the Guidezilla II Catheter
- The 5 and 5.5F sizes provide lower crossing profiles to support small vessel delivery
- The 5.5F size has the same effective I.D.* (inner diameter) as the 6F Guidezilla II Catheter[6] (this is outlined in the image below). This provides the ability to deliver the same devices as the 6F Guidezilla II Catheter with a smaller crossing profile

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension

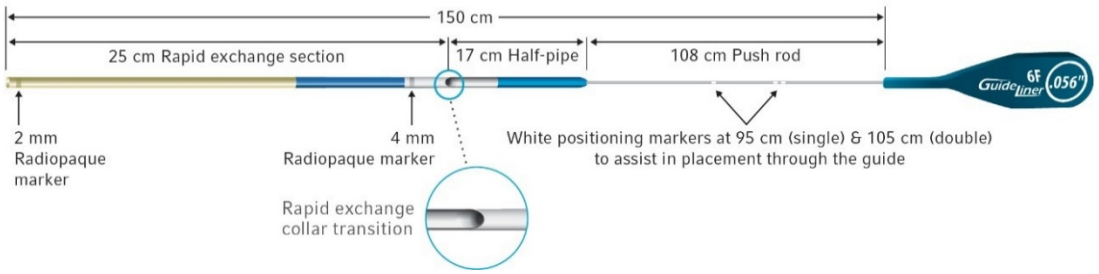
Effective I.D. Comparisons[7]



6. Dimensions taken from manufacturer's labeling and internal testing by Teleflex. Data on file.

Positioning and Value Statement

Positioning	• First in class, but currently competes against Guidezilla II Catheter. • Utilised when PCI operator anticipates the need to use guide extension to achieve: ○ Coaxial alignment and backup support ○ Facilitated distal device delivery ○ Selective (reduced) contrast delivery
Value Statement	The patented GuideLiner Catheter guide extension technology was first-to-market and developed in response for a convenient method to increase backup support and promote atraumatic deep-seating in percutaneous coronary interventions (PCI). The GuideLiner V3 Catheter, which offers a rapid exchange platform: • Utilises an intentional coil-reinforced, silicone-coated guide extension design that enables coaxial alignment and backup support. • Has the ability to deep-seat within a vessel for distal device delivery, and selective (reduced) delivery of contrast.



GuideLiner V3 Catheter

GuideLiner Catheters are intended to be used in conjunction with guide catheters to access discrete regions of the coronary and/or peripheral vasculature, and to facilitate placement of interventional devices.

PRODUCT NO.	SIZE	REQUIRED GUIDE CATHETER I.D.	PUSH ROD O.D.	GUIDE EXTENSION I.D.	TIP O.D.	RAPID EXCHANGE LENGTH	WORKING LENGTH
5569	5 Fr.	5 Fr. I.D. ≥0.056" (1.42 mm)	0.010" (0.25 mm)	0.046" (1.17 mm)	4.0 Fr. (1.35 mm / 0.053")	25 cm	150 cm
5570	5.5 Fr.	6 Fr. I.D. ≥0.066" (1.68 mm)	0.012" (0.30 mm)	0.051" (1.30 mm)	4.8 Fr. (1.60 mm / 0.063")	25 cm	150 cm
5571	6 Fr.	6 Fr. I.D. ≥0.070" (1.78 mm)	0.012" (0.30 mm)	0.056" (1.42 mm)	5.1 Fr. (1.70 mm / 0.067")	25 cm	150 cm
5572	7 Fr.	7 Fr. I.D. ≥0.078" (1.98 mm)	0.012" (0.30 mm)	0.062" (1.57 mm)	5.7 Fr. (1.90 mm / 0.075")	25 cm	150 cm
5573	8 Fr.	8 Fr. I.D. ≥0.088" (2.24 mm)	0.012" (0.30 mm)	0.071" (1.80 mm)	6.5 Fr. (2.16 mm / 0.085")	25 cm	150 cm

Packaged in quantities of 1 unit per box.

GuideLiner V3 Catheter Device Deployment and Handling Steps

Step	Action
1	**Catheter Insertion:** • Secure the previously inserted guidewire and backload the distal tip of the GuideLiner V3 Catheter onto the guidewire and advance until the catheter is just proximal to the haemostasis valve. • Open the haemostasis valve and advance the GuideLiner V3 Catheter through the haemostasis valve and into the guide catheter.
2	**Catheter Advancement:** • Under fluoroscopy, advance the GuideLiner V3 Catheter beyond the distal tip of the guide catheter and into the desired location within the vessel. ○ Confirm the desired position of the GuideLiner V3 Catheter in the vessel using fluoroscopy. **WARNING: Never advance the GuideLiner V3 Catheter into a vessel with an effective diameter less than 2.5mm. Vessel injury, ischemia, and/or occlusion may result. If pressure in a vessel dampens after inserting the GuideLiner V3 Catheter, withdraw the catheter until the pressure returns to normal.** **WARNING: Due to the size and non-tapered tip of the GuideLiner V3 Catheter, extreme care must be taken to avoid vessel occlusion and damage to the wall of the vessels through which this catheter passes.**
3	**Interventional Device Delivery:** • If performing an interventional procedure, backload the interventional device over the existing guidewire and advance the device through the guide catheter and GuideLiner V3 Catheter into the desired vascular space. • Tighten the Y-adaptor haemostasis valve securely on the proximal shaft of the GuideLiner V3 Catheter to prevent backbleeding. **NOTE: If a second wire is used during the intervention and encounters resistance within the guide catheter, pull the wire back several centimeters and slowly re-advance.**
4	**Catheter Removal:** • Perform the catheterisation procedure. • After completing the procedure, remove the GuideLiner V3 Catheter prior to removing the guide catheter from the vessel.

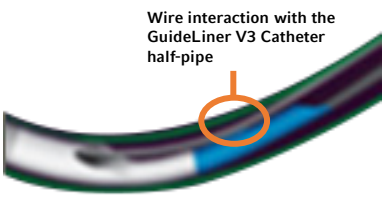
GuideLiner V3 Catheter Troubleshooting

Device won't advance

- If an operator is having difficulty introducing/advancing a second guidewire into the GuideLiner V3 Catheter, a microcatheter can be used to introduce the wire and reduce the chance of the wire getting caught in the GuideLiner V3 Catheter device collar. Once the wire is positioned within the guide extension segment, the microcatheter can be removed.

Device will not advance through the guide extension segment

- It is possible the device may be caught on the GuideLiner V3 Catheter pushrod wire. To avoid wire interactions/wrapping after the GuideLiner V3 Catheter has been positioned within the guide catheter, move the tab of the GuideLiner V3 Catheter pushrod away from the other wires and place a cloth over it to secure it in place.





Stent interaction

- Do not withdraw an undeployed stent back into the GuideLiner V3 Catheter when the catheter is in the body, as this action may damage, dislodge or embolise the stent. Instead, simultaneously pull both the GuideLiner V3 Catheter and undeployed stent back into the guide and remove together.

Device is difficult to remove

- Never advance the GuideLiner V3 Catheter more than 10cm beyond the tip of the guide catheter. The GuideLiner Catheter may become lodged in the guide catheter making it difficult to remove.

Troubleshooting Notes

GuideLiner V3 Catheter Competitive Overview

Competitive Device	Manufacturer	Sizes	Construction	Lengths (cm)	Effective I.D.*[6]	Tip O.D.	Key Features
Guidezilla II[7]	**Boston Scientific**	6F 6F Long 7F 8F	• Braid-reinforced guide extension • Circular pushrod • 6 mm hypotube collar transition • Radiopaque marker bands	Working length: 150 cm Rapid exchange: 25 cm (40 cm on 6F Long)	6F: 0.051″ 7F: 0.059″ 8F: 0.069″	6F: 0.067″ 6F Long: 0.067″ 7F: 0.073″ 8F: 0.083″	• Hydrophilic guide extension • Hypotube pushrod • Braided guide extension

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension segment.

6. Dimensions taken from manufacturer's labeling and internal testing by Teleflex. Data on file.
7. http://www.bostonscientific.com/content/gwc/en-US/products/catheters--guide/guidezilla-II-guide-extension-catheter.html. Website accessed 04/2019

GuideLiner V3 Catheter - Three Effective Techniques

✓ **Differentiate Device Construction and Performance**

- The GuideLiner V3 Catheter utilises a coil-reinforced guide extension design which offers greater longitudinal flexibility, superior shape retention in tortuosity or acute bends and is less kinkable than the Guidezilla II Catheter's braid-reinforced design[2].
- The GuideLiner V3 Catheter and its silicone-coated guide extension provides 18% greater backup support than the Guidezilla II Catheter and its fully hydrophilic-coated extension[8].
- The GuideLiner V3 Catheter's unique half-pipe channel creates a 17-centimeter polymer bond between the extension and the pushrod, an innovative feature designed to minimise device separation. The half-pipe channel directs and aligns devices through the collar transition, facilitating smooth device entry and seamless delivery.

✓ **Promote the Additional Size Offering**

- The GuideLiner V3 Catheter is available in 5F and 5.5F sizes beyond the 6F, 7F, and 8F sizes currently offered in the Guidezilla II Catheter.
- Not only do the 5F and 5.5F sizes provide a lower crossing profile that supports small vessel delivery, the 5.5F option has the same effective I.D.* as the 6F Guidezilla II Catheter[6]. This provides the ability to deliver the same devices as a 6F Guidezilla II Catheter with a smaller crossing profile.

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D. of the guide extension segment.

✓ **Promote Larger Effective I.D.**

- The GuideLiner V3 Catheter incorporates a flat pushrod that occupies less space than the circular pushrod utilised in the Guidezilla II Catheter. The flat pushrod of the GuideLiner V3 Catheter provides a larger effective I.D. within compatible guide catheters.

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D. of the guide extension segment.

2. All values and relative product comparisons are based on bench test data averages (n≥5) of competitive models and are analysed with a minimum of 95% confidence as a percentage of the means. Bench test results may not necessarily be indicative of clinical performance. Testing completed by Teleflex. Data on file.
6. Dimensions taken from manufacturer's labeling and internal testing by Teleflex. Data on file.
8. All values based on bench test data averages (n≥5) of 6F. models and are analysed with a minimum of 99% confidence as a percentage of the means.

Introduction to TrapLiner Catheter

What is Trapping?

"Trapping" was developed in response to a need to remove/exchange longer over-the-wire (OTW) equipment, such as a microcatheter, while maintaining distal guidewire positioning. During a coronary intervention, it can be difficult to exchange OTW catheters over shorter (e.g. 190 or 200cm) guidewires because of the risk of running out of wire on the proximal end and/or the risk of losing wire positioning or advancing the guidewire distally.

Trapping with a TrapLiner Catheter is one of four techniques utilised for removing OTW equipment without losing distal guidewire positioning. The alternative techniques, along with their advantages and disadvantages, are:

Guidewire Extensions

Coronary guidewires can be elongated by attaching a guidewire extension, also referred to as a dockwire. This technique requires the use of a matching proprietary (manufacturer-specific) extension, which is carefully fitted/attached to the exposed back end of the existing guidewire without kinking either component.

Removal or introduction of OTW equipment must then be performed under fluoroscopy (more radiation), however, even with careful use, guidewire position is easily lost. Coordination between the primary operator and an assistant is generally required.

Hydraulic Removal ("surfing", Nanto Technique)

Application of hydrostatic pressure into the lumen of a microcatheter via the proximal introducer port can be used to 'blow' (surf) the microcatheter out of the guide catheter and off the guidewire. This technique can be efficient when hand injection is sufficient (less efficient when an indeflator is required to generate sufficient pressure).

One of the disadvantages of this method, however, is that it does not actively stabilise the guidewire; the hydraulic force tends to advance the guidewire in the coronary, which can be hazardous (potential to cause dissections or perforations). Additionally, the distal guidewire position can be lost if traction on the OTW device momentarily exceeds the hydraulic effect.

While this technique does not require additional space within the guide catheter, this method only permits device removal – it does not aid device insertion.

Standard Trapping

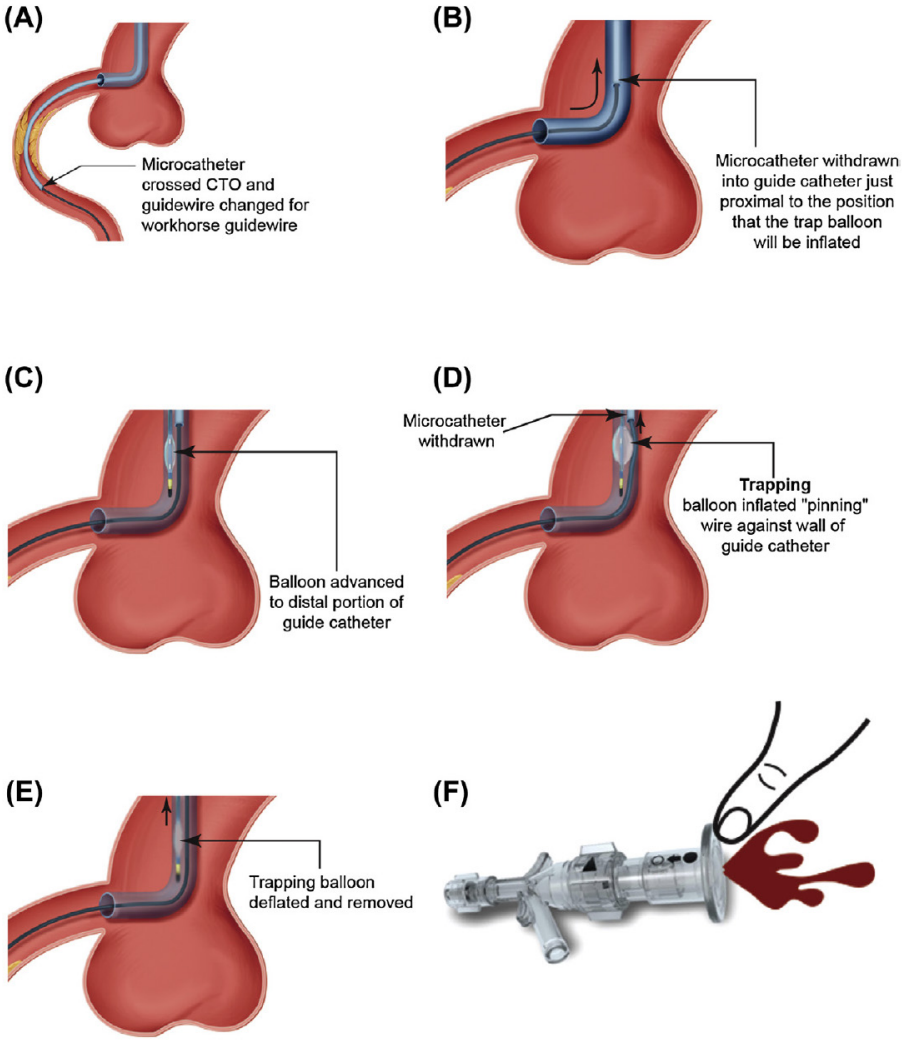
Standard trapping optimally stabilises the guidewire for OTW device removal or insertion; guidewire movement, much less loss of distal guidewire positioning, is rare. Typically, a PTCA balloon (2.5mm to 3.0mm nominal diameter) or a dedicated trapping balloon is placed distally within the guide catheter alongside the OTW microcatheter.

- Once the PTCA/dedicated trapping balloon is positioned – and while leaving the guidewire in its distal position – the OTW device is withdrawn until its distal tip is proximal to the PTCA/dedicated trapping balloon
- The PTCA/dedicated trapping balloon is then inflated within the guide catheter to 'trap' the guidewire against the inner guide catheter wall
- The OTW device is then fully removed
- The PTCA/dedicated trapping balloon is deflated, and any air that entered during device removal must bleed back through the proximal Y-connector valve.

Standard trapping creates space limitations within the guide catheter and potentially adds cost to the procedure by having to pull a separate PCTA/dedicated trapping balloon.

TrapLiner Catheter Illustration[9]

The image below depicts the standard trapping technique.



Summary of the Trapping Technique Within PCI:

Technique used to help facilitate exchanges of longer, OTW equipment (e.g. microcatheters) over shorter guidewires while maintaining guidewire position.

9. Image source: Brilakis, E. (2017). Manual of chronic total occlusion Interventions: a step-by-step approach, 2nd edition. Academic Press. Permission for use on file.

TrapLiner Catheter Overview

Product Description

Complex cases that require the trapping technique often additionally require the use of guide extension for added backup support and coaxial alignment.

The TrapLiner Catheter, which provides the combined benefits of guide extension and trapping capabilities, is the only 2-in-1 device currently available on the market.

The TrapLiner Catheter offers a rapid exchange platform that combines the ability to provide backup support along with the ability to trap a 0.014" guidewire against the inner wall of a guide catheter. This functionality helps prevent the loss of guidewire position or unintentional guidewire advancement during catheter exchange, which are common challenges during complex PCI procedures.

Because the TrapLiner Catheter has a shorter guide extension, it enables the ability to trap 18cm from the distal tip rather than forcing operators to trap proximal to the GuideLiner V3 Catheter guide extension collar.

The TrapLiner Catheter additionally eliminates the need for alternative catheter exchange techniques, such as the Nanto and standard trapping techniques. While effective, the loss of guidewire positioning is common with the Nanto technique and the method can only be used to 'trap out'/remove devices. Additionally, the standard trapping technique results in space limitations within the guide catheter and potentially adds cost to the procedure by having to pull a separate balloon just for trapping.

Indications and Contraindications

- The TrapLiner Catheter is intended for use in conjunction with guide catheters to access discrete regions of the coronary and/or peripheral vasculature, to facilitate placement of interventional devices, and to facilitate the exchange of an interventional device while maintaining the position of a guidewire within the vasculature.
- TrapLiner Catheter is contraindicated in vessels smaller than its tip outer diameter and in vessels in the neurovasculature.

Features & Benefits

The TrapLiner Catheter offers the following feature and benefits:

Feature: Integrated trapping balloon combined with guide extension

Benefits:

- Enables OTW catheter exchange over short (190 or 200 cm) guidewires with combination of guide extension and integrated trapping balloon
- Designed to offer convenience to procedure
- Designed for increased effective I.D.* in guide catheter compared to traditional trapping techniques
- Avoids difficulties of trapping with standard guide extension catheters

Benefits are expanded upon in further detail below.

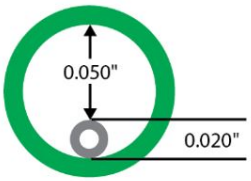
*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension segment.

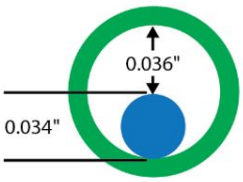
Benefit	Description
Enables OTW catheter exchange over short (190 or 200 cm) guidewires	• Preference of most PCI operators is to use short wires. • Integrated balloon on the TrapLiner Catheter helps secure the guidewire position during catheter exchanges by trapping the guidewire against the inner wall of the guide catheter. • Only guide extension device currently available that combines the benefits of guide extension with an integrated 0.014" guidewire trapping solution. ○ Complex cases often require the use of guide extension for added backup support and coaxial alignment.
Designed to offer convenience to a procedure	• Designed to save time and effort and eliminates the need to repeatedly deliver off-the-shelf trapping balloons for OTW catheter exchanges. ○ Potential opportunity to reduce procedure time and radiation exposure, especially when multiple exchanges are needed in a procedure.

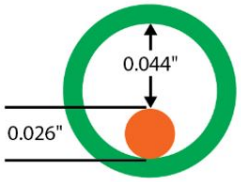
<table>
<tr><th>Benefit</th><th>Description</th></tr>
<tr><td>Designed for increased effective I.D.* in guide catheter compared to traditional trapping techniques

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension portion.</td><td>

While the round hypotube pushrod is larger than the flat GuideLiner V3 Catheter pushrod, the TrapLiner Catheter pushrod requires less space in the guide when compared to the space other trapping techniques require:

6F TrapLiner® Catheter
Trapping = Yes
Guide Extension = Yes
Push Rod O.D. = 0.020”
Effective I.D.[10] = 0.050”
 

PTCA Balloon Example
Trapping = Yes
Guide Extension = No
Avg. O.D.[11] ≈ 0.034”
Effective I.D.[10] ≈ 0.036”
 

Trapper® Catheter
Trapping = Yes
Guide Extension = No
Device O.D.[12] ≈ 0.026”
Effective I.D.[10] ≈ 0.044”
 

6F platform provides the ability to use a microcatheter while also having both guide extension and trapping capabilities. The following are examples of devices that are compatible in a 6F TrapLiner Catheter+

Turnpike Microcatheters (0.038”)
Twin-Pass Dual Access Catheters (0.046”)
Stingray LP Coronary CTO Re-Entry System (0.042”)
CrossBoss Coronary CTO Crossing Catheter (0.045”)

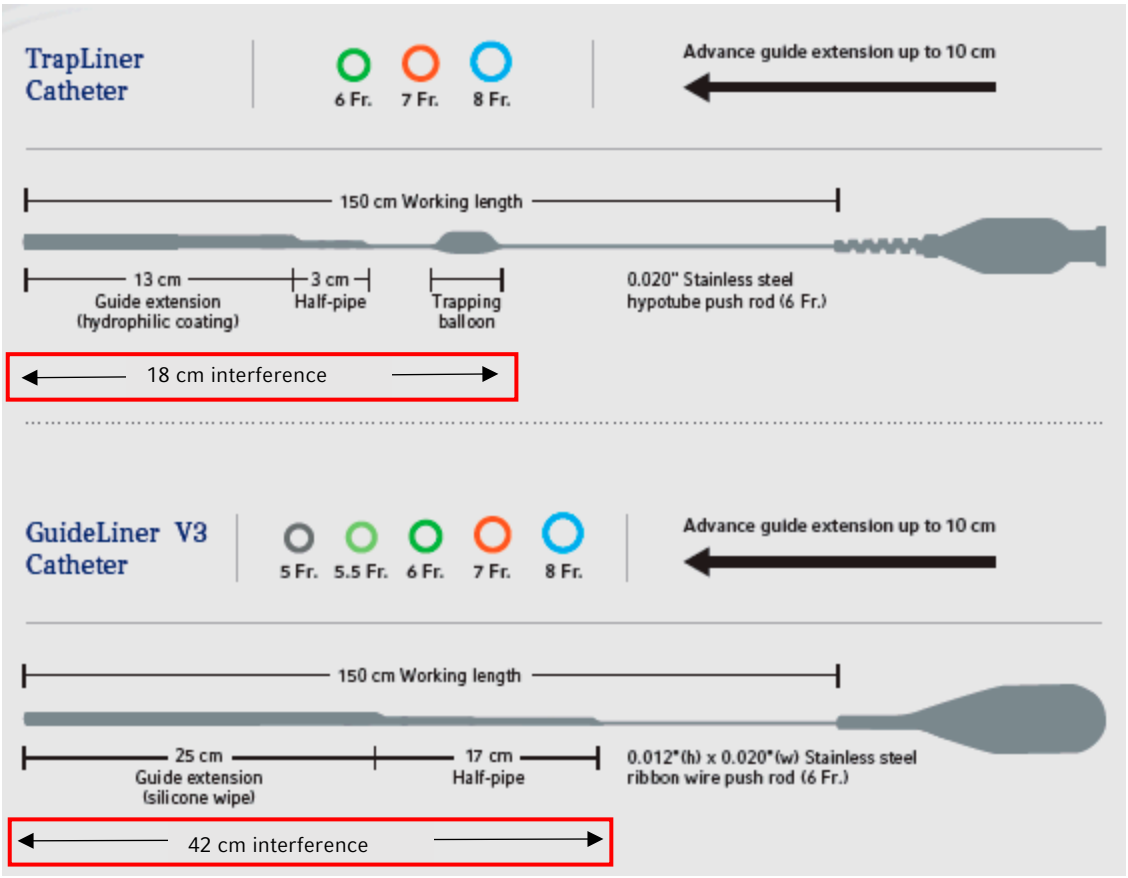
+ Device compatibility based on marketed proximal shaft outer diameter measurements deployed within a 6F (0.070”) guide catheter, and not simulated use testing.
</td></tr>
<tr><td>Avoids difficulties of trapping with standard guide extension catheters</td><td>

Shorter guide extension enables trapping 18cm from the distal tip. Standard guide extension catheters force operators to trap proximal to the guide extension collar.
Designed to reduce the incidence of running out of proximal guidewire.

</td></tr>
</table>

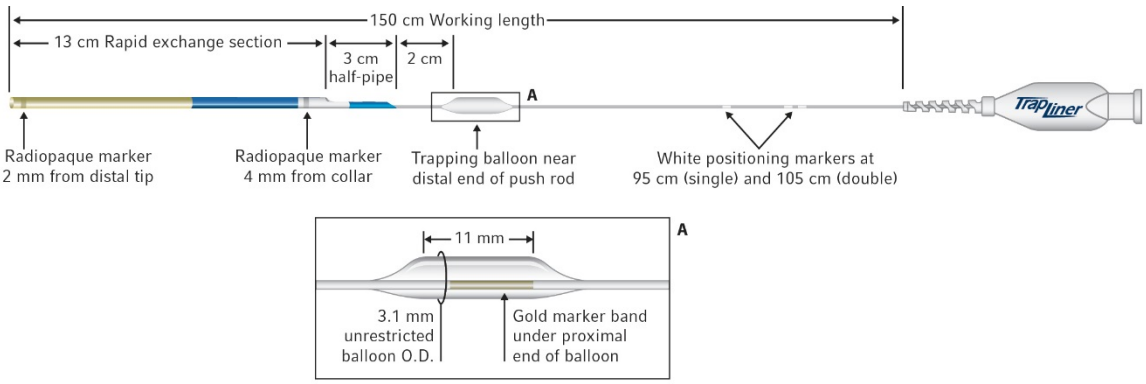
10. Based on a 6F (0.070”) guide catheter I.D.
11. This O.D. represents the average measured on commonly used PTCA balloons in the diameter range of 2.25 to 2.75 mm. Maximum O.D.s on PTCA balloon catheters at a specific diameter will show variance across different manufacturers, versions, and lots.
12. Device O.D. based on manufacturer labeling

Feature Comparison Overview



Positioning and Value Statement

Positioning	• Currently no direct competitor. • The TrapLiner Catheter is a 2-in-1 device. ○ Offers the combined benefits of guide extension and trapping capabilities. • Product should be utilised if PCI operator anticipates the need to use guide extension and has the potential to trap during a procedure.
Value Statement	The TrapLiner Catheter is the only guide extension device that combines the benefits of guide extension with an integrated 0.014" guidewire trapping solution, designed to offer more space in the guide to utilise both guide extension and a microcatheter, all while having trapping capabilities.



TrapLiner Catheter

The TrapLiner Catheter is intended for use in conjunction with guide catheters to access discrete regions of the coronary and/or peripheral vasculature, to facilitate placement of interventional devices, and to facilitate the exchange of an interventional device while maintaining the position of a guidewire within the vasculature.

MODEL	SIZE	MINIMUM GUIDE CATHETER SIZE	MAXIMUM GUIDE CATHETER SIZE	PUSH ROD O.D.	GUIDE EXTENSION I.D.	GUIDE EXTENSION O.D.	RAPID EXCHANGE LENGTH	NOMINAL PRESSURE RATING	RATED BURST PRESSURE
5566	6 Fr.	6 Fr. (0.070" / 1.78 mm I.D.)	8 Fr. (0.091" / 2.31 mm I.D.)	0.020" (0.51 mm)	0.056" (1.42 mm)	5.1 Fr. (1.70 mm / 0.067")	13 cm	12 atm / 1216 kPa	14 atm / 1419 kPa
5567	7 Fr.	7 Fr. (0.078" / 1.98 mm I.D.)	8 Fr. (0.091" / 2.31 mm I.D.)	0.025" (0.64 mm)	0.062" (1.57 mm)	5.7 Fr. (1.90 mm / 0.075")	13 cm	12 atm / 1216 kPa	14 atm / 1419 kPa
5568	8 Fr.	8 Fr. (0.088" / 2.24 mm I.D.)	8 Fr. (0.091" / 2.31 mm I.D.)	0.025" (0.64 mm)	0.071" (1.80 mm)	6.5 Fr. (2.16 mm / 0.085")	13 cm	12 atm / 1216 kPa	14 atm / 1419 kPa

Packaged in quantities of 1 unit per box.

TrapLiner Catheter Deployment and Handling Steps

Step	Action
1	**Catheter Insertion:** • Secure the previously placed guidewire. Backload the distal tip of the TrapLiner Catheter onto the proximal end of the guidewire. • Advance through the haemostasis valve and into the guide catheter. **TROUBLESHOOTING TIP: Ensure the haemostasis valve is open prior to delivery and hold the catheter at the extension segment – NOT the hypotube – until the full extension has been delivered into the haemostasis valve.**
2	**Catheter Advancement:** • Under fluoroscopy, advance the TrapLiner Catheter beyond the distal tip of the guide catheter and into the desired location within the vessel.
3	**Interventional Device Delivery:** • If performing an interventional procedure, backload the interventional device over the existing guidewire and advance the device through the guide catheter and TrapLiner Catheter and into the desired vascular space. **NOTE: If a second wire is used during the intervention and encounters resistance within the guide catheter, pull the wire back several centimeters and re-advance.**
4	**Trapping Out:** • If trapping of the guidewire within the guide catheter for a device exchange is desired, withdraw the previously placed catheter over the existing 0.014" guidewire until the distal tip of the catheter is proximal to the radiopaque marker under the TrapLiner Catheter balloon. • Confirm position using fluoroscopy. • Inflate the TrapLiner Catheter balloon using an inflation device and slowly remove the catheter out of the guide catheter. **Trapping In:** • If desired, a new catheter can be loaded onto the 0.014" guidewire and advanced until the catheter tip approaches the TrapLiner Catheter balloon. • Deflate the balloon under fluoroscopy and allow for bleedback from the haemostasis valve prior to advancing the catheter.
5	**Catheter Removal:** • After completing the procedure, remove the TrapLiner Catheter prior to removing the guide catheter from the vessel.

TrapLiner Catheter Troubleshooting

Proper handling techniques

- Coiling the balloon-collar section of the catheter
  - You may coil the catheter on the table or in a bowl of saline before/after use. Do NOT coil backbone at or distal to balloon.
- Delivery into haemostasis valve
  - Ensure haemostasis valve is OPEN prior to delivery.
  - Grasp the catheter on the extension segment – NOT the hypotube – until full extension has been delivered into the haemostasis valve.
- Wetting catheter in bowl of saline
  - Be cognisant of the weld junction between the balloon and the collar.
    - Lay extension segment flat into bowl to wet – do NOT force the catheter face down.
- Advancing collar junction out of guide
  - Operators must be mindful to the proximal marker band (4mm distal to collar) to avoid 'over-extension'. Never advance the TrapLiner Catheter more than 10cm beyond the tip of the guide.
  - Exposure of balloon-collar segment to vasculature outside of the guide presents increased risk of kink or separation, as this segment was designed to be supported by a guide catheter.
- Backbone is NOT as robust as GuideLiner V3 Catheter and should be handled like a trapping device.
  - TrapLiner Catheter backbone requires much more delicate handling than GuideLiner V3 Catheter **to reduce the risk of kink or separation.**
  - TrapLiner Catheter hypotube (hollow core) is more susceptible to kinking vs. the GuideLiner V3 Catheter (ribbon wire).
  - "Kink point" = point on catheter located 1-2 mm proximal to half-pipe that is most prone to kinking (*corresponds to weld junction on distal hypotube*).

Where is the balloon located (gold marker)?	• The balloon is located 18cm from the distal end of the guide extension. The balloon is 11mm and has a gold marker band at the proximal end of the balloon. Depending on how far the guide extension segment is advanced outside of the guide catheter, the balloon is often positioned in the descending aorta.
Device won't advance through the guide extension segment	• To avoid wire wrapping, achieve placement with your guide extension and move the pushrod away from the other wires. Place a cloth over the pushrod to secure it.

Troubleshooting Notes

_____

_____

_____

_____

_____

_____

_____

_____

TrapLiner Catheter Competitive Overview

There are no direct TrapLiner Catheter competitors on the market, as there currently are no other guide extension catheters that have an integrated trapping balloon. There are, however, guide extension catheters, trapping devices and balloon catheters with varying sizes on the market.

Competitive Device	Manufacturer	Balloon Size	Markerband Location	Balloon Pressure Specs (atm)	Key Features
Trapper[13]	**Boston Scientific**	• 10 mm balloon length • 2.4 mm at nominal, uncoated (non-compliant) • 6 – 8F compatible	• 2 mm long and double thickness (of POBA) markerband • 1 mm long distal markerband to confirm position in guide catheter	• Nominal: 12 atm • Rated Burst Pressure: 20 atm	• Compatible with 90 or 100 cm guide catheters • Compatible with 6F, 7F and 8F guide catheters • Low-profile shaft (2F diameter) • Core wire tip to hub for durability during multiple uses

13. http://www.bostonscientific.com/content/gwc/en-US/products/exchange-device/trapper-exchange-device.html. Website accessed 04/2019

TrapLiner Catheter Competitive Messaging

Competitive Device	Manufacturer/ Distributor (US)	Primary Competitive Messaging
Guidezilla II[7]	Boston Scientific	• Guidezilla II Catheter is an excellent supportive device for delivery of interventional devices: ○ Braid-reinforced guide extension for excellent pushability. ○ Hydrophilic guide extension coating improves deliverability. • By supporting the delivery of the interventional devices that are necessary to complete a procedure, Guidezilla II Catheter can benefit the patient and the hospital by turning an unsuccessful PCI into a successful PCI. • Guidezilla Catheter utilises a short hypotube transition to reduce device interactions. • Guidezilla II Catheter has an expanded size matrix offering; available in 6F, 7F, 8F and 6F Long.
Trapper[13]	Boston Scientific	• Treat efficiently in a 6 – 8F guide for maximum utility. • Trap confidently with a high-pressure device that is recognisable at a glance. • Repeat multiple times leveraging a tip-to-hub core wire for durability. • A fixed wire catheter that has a 10 mm balloon near the distal tip, designed to secure a 0.014" wire within a 6F through 8F guide catheter when inflated to 12 ATM. • 1 mm marker band at the tip and a 2 mm marker band at the proximal balloon edge to facilitate easy placement and identification during rapid exchanges.

7. http://www.bostonscientific.com/content/gwc/en-US/products/catheters--guide/guidezilla-II-guide-extension-catheter.html. Website accessed 04/2019
13. http://www.bostonscientific.com/content/gwc/en-US/products/exchange-device/trapper-exchange-device.html. Website accessed 04/2019

TrapLiner Catheter – Three Effective Techniques

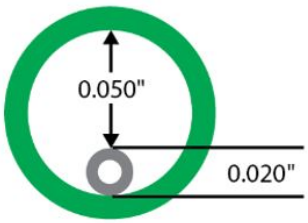
✓ **Promote the Benefits of a 2-in-1 Device**

- The TrapLiner Catheter is a rapid exchange guide extension catheter that combines the ability to provide backup support with the ability to trap a 0.014" guidewire against the inner wall of a guide catheter.

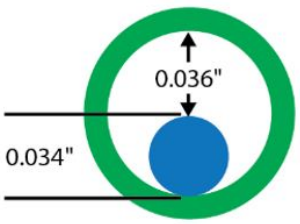
✓ **Promote Larger Effective I.D.***

- The TrapLiner Catheter round hypotube pushrod requires less space in the guide when compared to the effective I.D. space other trapping techniques require

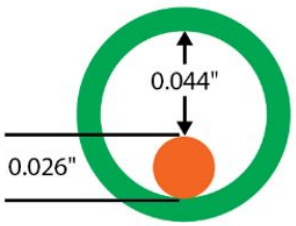
6F TrapLiner® Catheter
Trapping = Yes
Guide Extension = Yes
Push Rod O.D. = 0.020"
Effective I.D.[10] = 0.050"



PTCA Balloon Example
Trapping = Yes
Guide Extension = No
Avg. O.D. [11] ≈ 0.034"
Effective I.D.[10] ≈ 0.036"



Trapper® Catheter
Trapping = Yes
Guide Extension = No
Device O.D.[12] ≈ 0.026"
Effective I.D.[10] ≈ 0.044"



- The TrapLiner Catheter 6F platform provides the ability to use a microcatheter while also having both guide extension and trapping capabilities

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D. of the guide extension segment.

✓ **Optimised Guide Extension length for Trapping**

- The TrapLiner Catheter has a shorter guide extension to enable trapping 18cm from the distal tip. Standard guide extension catheters force operators to trap proximal to the guide extension collar
- The TrapLiner Catheter was designed to reduce the incidence of running out of proximal guidewire

10. Based on a 6F (0.070") guide catheter I.D.
11. This O.D. represents the average measured on commonly used PTCA balloons in the diameter range of 2.25 to 2.75 mm. Maximum O.D.s on PTCA balloon catheters at a specific diameter will show variance across different manufacturers, versions, and lots.
12. Device O.D. based on manufacturer labeling

Effective Targeting

The primary target for the GuideLiner V3 Catheter and the TrapLiner Catheter is the cath lab where PCI procedures are performed.

The GuideLiner V3 Catheter and TrapLiner Catheters are indicated for peripheral vasculature interventions, but are not as readily utilised in the peripheral vascular space. This is because physicians usually have a robust, coaxial support system from larger guiding sheaths (e.g. Super Arrow-Flex Sheaths) which usually provide adequate device support in the lower extremities.

Additionally, physicians do not have to negotiate the aortic root transition to the smaller coronary arteries, which often poses challenges with guide back-out and support.

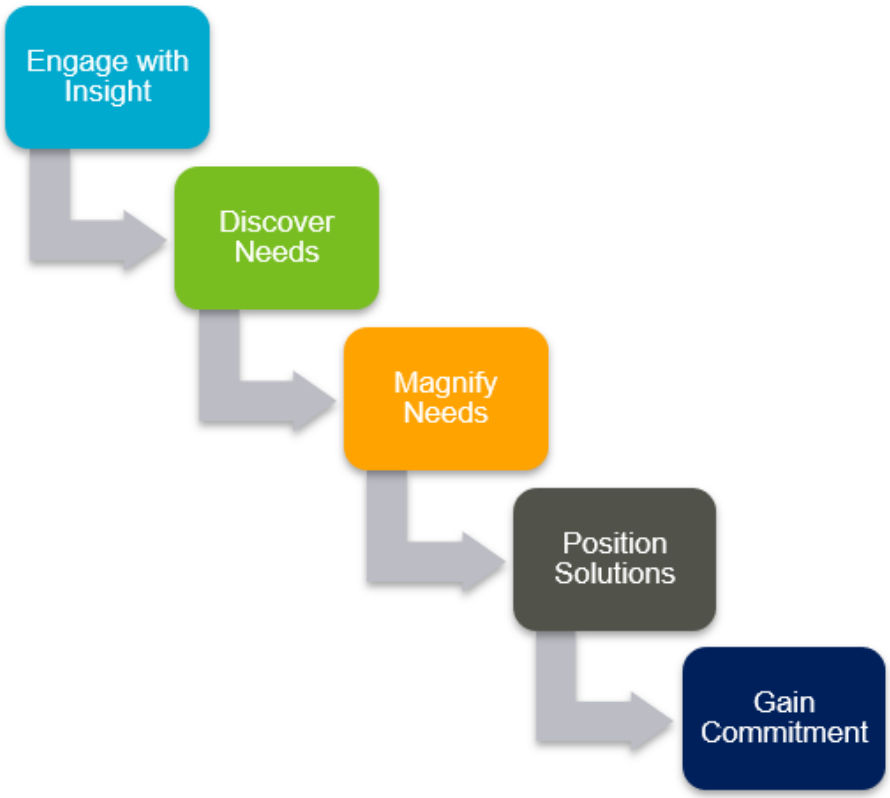
Interventional Cardiologists (ICs)

- ICs are the primary clinical target for these products
- Most efficient targets are ICs who consistently perform complex procedures that require the benefits of guide extension (coaxial alignment and backup support, deep seating for distal device delivery and selective delivery of contrast), but also will benefit from a more efficient form of trapping.

For more specific territory and regional targeting, contact your Regional Manager to obtain a copy of the PCI targeting tool.

Applying the Balanced Seller Process

The Balanced Seller Process



Pre-call Planning

Completing a pre-call plan is vital to the success of a sales call. This is the time to crystallise your goal and focus on that specific customer and their needs.

Below are the focus areas of a pre-call plan, with examples pre-populated for your reference.

Question	Response
Background	
Who are you seeing today?	Dr. George
How did this meeting come about?	Scheduled lab day with lunch in service
About the customer	
What is their area of interest / specialty / role?	Coronary Interventions/ Complex PCI / Interventional Cardiologist
Hospital or department priorities and initiatives?	Priorities – Develop and grow complex procedure program Initiatives – Improve efficiencies while maintaining high-quality patient care
Past education training / plans implemented?	Multiple lunch in-services on GuideLiner V3 Catheter, CEU presentations given on Haemostasis & Vascular Access
Current product usage?	Guidezilla II Catheter, GuideLiner V3 Catheter, Trapper Exchange Device
Key people to talk to? Who is on the team?	Scrub Tech – Dave; Nurse Manager – Gabby; Purchasing – Dan
Call Plan	
Objectives for the meeting:	
What action do you want the customer to take to advance the sales process?	Agree to TrapLiner Catheter evaluation and contact purchasing to approve GuideLiner V3 Catheter 5.5F size purchase
What outcome are YOU looking for?	Gain order approval for 5.5F GuideLiner V3 Catheter and TrapLiner Catheter evaluation, identify new users
What resources / tools do you need to bring with you?	Product demos, brochures, digital tablet, coronary model w/ guide catheter and 0.014” guidewire

How will you engage the customer?	
Insight / Agenda / Outcome for customer	*"I understand you have been doing more interventions through a 6F system. I'd like to learn more about the challenges you've encountered and discuss how we might be able to address those challenges."*
What high impact questions will you ask?	**Double click questions?**
What is your current method of trapping?	What happens when you need to utilise guide extension, a microcatheter and need to trap within a 6F system?
What type of problems have you had when using guide extension?	How does that impact your approach to treating patients?
How does treating complex cases impact your daily schedule (office, other physicians, patients, hospital, etc.)?	How does that impact your team?

Balanced Seller Process: GuideLiner V3 Catheter and TrapLiner Catheter

Engage with Insight



To demonstrate insight and position the meeting:

- Insight: Customer priority to explore
- Agenda: What you intend to do
- Outcome: What is in it for the customer

Engage Examples:

- *"I understand you have been doing more interventions through a 6F system. I'd like to learn more about what challenges you've encountered and discuss how we may be able to help."*
- *"I was on the hospital website and saw that innovation is a large part of your vision statement. I'd like to learn more about what your department is doing and how Teleflex can assist in aligning with that vision."*

Discover Needs 4 "P" Question



To build a clear, complete and mutual understanding of a customer's needs:

- Open High Impact Questions
- Questioning strategy: 4 Ps
- Double Click

Personal

- *What type of procedures are you most passionate about?*
- *What is your role during complex PCI cases?*
- *What role do you play in bringing new products into the lab?*

Process

- *What are the biggest challenges during complex procedures?*
- *What issues do you encounter while using guide extension and trapping?*
- *How do you track efficiencies and radiation exposure?*

Procedure

- *What is your current method of trapping with guide extension?*
  ***Double Click** What happens when you're using a 6F system and you need to use a microcatheter and guide extension?*
- *What type of problems have you had when using guide extension?*
  ***Double Click** How does that affect your approach to treating patients?*

People

- *Who is responsible for approving new product?*
- *How does the lab manager react when cases run longer than expected?*
- *Who is the Chief Interventional Fellow?*



Magnify Needs

Magnify Needs involves creating the problem or need big enough that the customer is willing to act

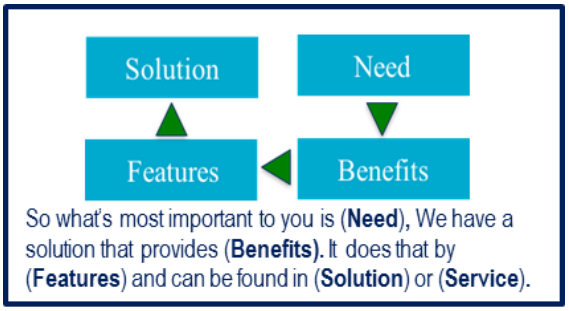
- Ask implication or impact questions to determine cause and effect
- Confirm the customer is willing to solve the problem

- *How much does that affect procedure time and cost?*
- *What is the consequence of this problem?*
- *How does that impact your team?*

Position Solutions

To help a customer understand specifically how you can satisfy their need

- Put the customer in the picture
- Need – Benefit – Feature – Solution



GuideLiner V3 Catheter

So, *what you're saying is that smaller, more tortuous vessels can add difficulty to guide extension deliverability – overall, leading to frustrations and increased procedure time.*

(Check for agreement)

When you encounter those challenging vessels and need additional backup support, we have a solution that utilises a coil-reinforced, silicone-coated guide extension design. This design not only provides exceptional longitudinal flexibility and shape retention when navigating tortuous anatomy, it also is designed to provide the required backup support needed to complete those challenging procedures.

Additionally, our GuideLiner V3 Catheter product offering includes 5F and 5.5F sizes beyond our 6F, 7F, and 8F offerings. Not only do these smaller sizes provide a lower crossing profile that supports small vessel delivery, the 5.5F option has the same effective I.D. as certain competitive 6F devices[6], providing you with the ability to deliver the same devices within a smaller crossing profile.*

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension portion.

6. Dimensions taken from manufacturer's labeling and internal testing by Teleflex. Data on file.

TrapLiner Catheter

So, what you are saying is that when you need to trap during a procedure there is not enough space in the guide to utilise key equipment, such as microcatheter and guide extension, when using a 6F catheter.

(Check for agreement)

We have a solution when space is a limiting factor and you need to utilise all the tools at your disposal. By combining the benefits of guide extension and a trapping balloon, the TrapLiner Catheter provides you with more space within the guide [10, 11, 12] *so you can simultaneously utilise both guide extension and a microcatheter, all while having trapping capabilities.*

Not only does the TrapLiner Catheter deliver the combined benefits of guide extension and trapping capabilities, because it is a 2-in-1 device, you can reduce your overall product utilisation by incorporating the TrapLiner Catheter as your first option – potentially reducing overall product costs.

Gain Commitment



To agree with customer on best next steps

- Advancement or Yes

(Check for agreement)

Great! Can I bring one in for your next case?

10. Based on a 6F (0.070") guide catheter I.D.
11. This O.D. represents the average measured on commonly used PTCA balloons in the diameter range of 2.25 to 2.75 mm. Maximum O.D.s on PTCA balloon catheters at a specific diameter will show variance across different manufacturers, versions, and lots.
12. Device O.D. based on manufacturer labeling

Handling Objections and using CPR

- **C**larify the problem
- **P**araphrase the need
- **R**espond with facts and/or benefits

Test for agreement

GuideLiner V3 Catheter Handling Objections

Customer: *"Having smaller sizes sounds nice but I'm not sure it's worth taking up shelf space."*

Rep: (Clarify problem) *"What about the smaller sizes do you find appealing?*

Customer: *"It would be nice to have the options on hand if I'm working in a small vessel or using a smaller system, but that doesn't happen often."*

Rep: (Paraphrase the Need) *"So, having the right tool for the job while also providing the most utility is important to you?"*

Customer: *"Yes, I would agree with that."*

Rep: (Facts/benefits) *"When you encounter those challenging vessels and need additional backup support, we have a solution that utilises a coil-reinforced, silicone-coated guide extension design. This design not only provides exceptional longitudinal flexibility and shape retention when navigating tortuous anatomy, it also is designed to provide the required backup support needed to complete those challenging procedures.*

Additionally, our GuideLiner V3 Catheter product offering includes 5F and 5.5F sizes beyond our 6F, 7F, and 8F offerings. Not only do these smaller sizes provide a lower crossing profile that supports small vessel delivery, the 5.5F option has the same effective I.D. as certain competitive 6F devices[6], providing you with the ability to deliver the same devices within a smaller crossing profile."*

(Test for agreement) *"Does that provide enough utility?"*

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension segment.

6. Dimensions taken from manufacturer's labeling and internal testing by Teleflex. Data on file.

TrapLiner Catheter Handling Objections

Customer: *"We're happy with our current method of trapping and don't see the need for the TrapLiner Catheter."*

Rep: (Clarify problem) *"What is it you like about your current method of trapping?"*

Customer: *"It's simple, effective and cost efficient."*

Rep: (Paraphrase the Need *"So, am I correct in saying that having the right tool for the job while also ensuring cost effectiveness and efficiencies are most important to you and your practice."*

Customer: *"Yes, I would agree with that."*

Rep: (Facts/benefits) *"We have a solution when space is a limiting factor and you need to utilise all the tools at your disposal. By combining the benefits of guide extension and a trapping balloon, the TrapLiner Catheter provides you with more space within the guide*[10, 11, 12] *so you can simultaneously utilise both guide extension and a microcatheter, all while having trapping capabilities.*

Not only does the TrapLiner Catheter deliver the combined benefits of guide extension and trapping capabilities, because it is a 2-in-1 device, you can reduce your overall product utilisation by incorporating the TrapLiner Catheter as your first option – potentially reducing overall product costs."

(Test for agreement) *"Does that provide enough utility?"*

10. Based on a 6F (0.070") guide catheter I.D.
11. This O.D. represents the average measured on commonly used PTCA balloons in the diameter range of 2.25 to 2.75 mm. Maximum O.D.s on PTCA balloon catheters at a specific diameter will show variance across different manufacturers, versions, and lots.
12. Device O.D. based on manufacturer labeling

The Balanced Seller Process: GuideLiner V3 Catheter: Putting It All Together

Rep:	*"I understand you have been doing more interventions through a smaller system. I'd like to learn more about what challenges you've encountered and discuss how we may be able to assist in addressing those challenges."*
Dr. George	**"Yes, that's correct. I try to complete procedures using a 6F or smaller system as often as I can."**
Rep	*"What types of challenges do you encounter when using guide extension within a smaller system?"*
Dr. George	**"I have challenges getting my guide extension to go where I need it to go while still being able to deliver my devices."**
Rep	*"What kind of vessels are the most challenging?"*
Dr. George	**"Tourtous vessels for sure, and when I need to get deeper into the smaller vessels the guide extension can sometimes fight me."**
Rep	*"How does that impact the case?"*
Dr. George	**"It's frustrating. I spend more time then is necessary trying to complete the case."**
Rep	*"So, what you're saying is that smaller, more tortuous vessels can add difficulty to guide extension deliverability?*
Dr. George	**"Yes, that's correct."**
Rep	*"When you encounter those challenging vessels and need additional backup support, we have a solution that utilises a coil-reinforced, silicone-coated guide extension design. This design not only provides exceptional longitudinal flexibility and shape retention when navigating tortuous anatomy, it also is designed to provide the required backup support needed to complete those challenging procedures.* *Additionally, our GuideLiner V3 Catheter product offering includes 5F and 5.5F sizes beyond our 6F, 7F, and 8F offerings. Not only do these smaller sizes provide a lower crossing profile that supports small vessel delivery, the 5.5F option has the same effective I.D.* as certain competitive 6F devices[6], providing you with the ability to deliver the same devices within a smaller crossing profile."*
Rep	*"I have sterile samples if you'd like to try one on your next case. If it goes well, would you email the manager about setting up a formal evaluation?"*
Dr. George	**"Yes, that sounds good."**

*Effective I.D. represents the smallest applicable I.D. encountered within either the guide extension or the guide catheter. Note this is different than guide extension I.D. which only represents the I.D of the guide extension segment.

6. Dimensions taken from manufacturer's labeling and internal testing by Teleflex. Data on file.

The Balanced Seller Process: TrapLiner Catheter: Putting It All Together

Rep:	*"I understand you have been doing more interventions through a 6F system. I'd like to learn more about what challenges you've encountered and discuss how we may be able to assist in addressing those challenges."*
Dr. George	**"Yes, that's correct. I try and stay in a 6F system as much as I can."**
Rep	*"For those cases where you need to trap, how does using the smaller system factor into your decision on which products to use?"*
Dr. George	**"It's a big factor. I have limited space and need to be conscious of my effective I.D.*"**
Rep	*"Are there instances when the limited space results in you not being able to use a device?"*
Dr. George	**"Yes, I can't always use all the tools at my disposal."**
Rep	*"How does that impact the case?"*
Dr. George	**"I spend more time then is necessary trying to complete the case, or I am forced to upsize the guide catheter."**
Rep	*"So, what you are saying is that it can be frustrating when you need to trap during a procedure and there is not enough space in the guide to utilise key equipment, such as guide extension and a microcatheter, when using a 6F catheter?"*
Dr. George	**"Yes, that's correct."**
Rep	*We have a solution when space is a limiting factor and you need to utilise all the tools at your disposal. By combining the benefits of guide extension and a trapping balloon, the TrapLiner Catheter provides you with more space within the guide[10, 11, 12] so you can simultaneously utilise both guide extension and a microcatheter, all while having trapping capabilities.* *Not only does the TrapLiner Catheter deliver the combined benefits of guide extension and trapping capabilities, because it is a 2-in-1 device, you can reduce your overall product utilisation by incorporating the TrapLiner Catheter as your first option – potentially reducing overall product costs.*
Rep	*"I have a model and demo set up in the breakroom, if you'd like to try it."*
Dr. George	"Yes, that sounds good."

10. Based on a 6F (0.070") guide catheter I.D.
11. This O.D. represents the average measured on commonly used PTCA balloons in the diameter range of 2.25 to 2.75 mm. Maximum O.D.s on PTCA balloon catheters at a specific diameter will show variance across different manufacturers, versions, and lots.
12. Device O.D. based on manufacturer labeling

The Balanced Seller Process: YOU TRY IT

Practice: Discover Needs

Directions: Create high impact questions that would be appropriate for your customer. Consider all four Ps. Look to develop 4 good questions you could ask. For each question, list one double click question you could ask.

Personal

Double Click:

Process

Double Click:

Procedure

Double Click:

People

Double Click:

Discover Needs Magnified

Directions: Choose one of the needs you discovered and list questions to magnify that need.

Need you discovered:

Question to magnify the need:

Position Solutions

Directions: For your magnified need "put the customer in the picture" by writing what you would say to position a solution in the order below.

Need: ("*So, what's most important to you is…*")

Benefits: ("*We have a solution that…*")

Features: ("*It does that by…*")

Solution: *("…and it's called…*")

Gain Commitment

Directions: Using your solution from above, develop a response to gain commitment using the steps.

Summarise Benefits:

Propose Next Steps for Customer:

Next Steps for YOU:

Ask for Commitment:

Handling Objections

Directions: Develop a response that handles a common objection YOU have encountered in the field, using CPR.

Objection: _____

Clarify the problem:

_____

_____

_____

_____

_____

Paraphrase the NEED:

_____

_____

_____

_____

Respond (benefits/facts):

_____

_____

_____

_____

Test for Agreement:

Selling Tools and Resources

Running a Successful In-service

When given the opportunity to set up for a product in-service, it is crucial that you are prepared. In-services give you the opportunity to showcase multiple products, drive usage or move the sale forward.

Below is a list of product specific tools and resources to help you achieve this goal and make the most of the in-service.

GuideLiner V3 Catheter Suggested Materials Checklist

Materials	Explanation	✓
Coronary Model with Guide catheter	Utilise to demonstrate advancement through the guide catheter and into the ostium.	☐
Guardian Haemostasis Valve	Showcase Guardian Haemostasis Valve during the hands-on demonstration.	☐
0.14" coronary guidewires	Use Teleflex portfolio of coronary guidewires a bring multiple wires in case one becomes damaged.	☐
GuideLiner V3 Catheter Non-sterile demos	Good quality non-sterile demos; if your sample is damaged contact training to try and get a replacement	☐
Digital tablet or phone w/ GuideLiner V3 Catheter deployment video	Visual aid that highlights proper deployment of guide extension.	☐

TrapLiner Catheter Suggested Materials Checklist

Materials	Explanation	✓
Coronary Model with Guide catheter	Utilise to demonstrate: • Advancement through the guide catheter and into the ostium • Balloon location and how it enables OTW exchanges with short guidewires • Trapping in/out with microcatheter exchanges	☐
Clear Guide Catheter	Allows for visualisation of the balloon when demonstrating the product for operators. The clear guide catheter is compatible with a 6F TrapLiner Catheter.	☐
Guardian Haemostasis Valve	Showcase Guardian Haemostasis Valve during the hands-on demonstration. Be sure to show proper advancement techniques to avoid kinking.	☐
0.14" coronary guidewires	Use Teleflex portfolio of coronary guidewires and bring multiple wires in case one becomes damaged.	☐
GuideLiner V3 Catheter Non-sterile demos	Good quality non-sterile demos; if your sample is damaged contact training to try and get a replacement	☐
Turnpike Catheter Non-sterile demos	Good quality non-sterile demos; if your sample is damaged contact training to try and get a replacement.	☐
Digital tablet or phone w/TrapLiner Catheter deployment video	Visual aid that highlights proper deployment of trapping capabilities.	☐

Marketing Materials

There are many resources available for you to assist in selling the GuideLiner V3 Catheter and TrapLiner Catheter products. These materials range from demo models, brochures and digital animations showing how the products work.

GuideLiner V3 Catheter

Item	Location	Contact for More Information
Coronary Demo Model	Received after New Hire Training	Sales/Marketing Coordinator
GuideLiner V3 Catheter Product Brochure	SAM	Product Manager
GuideLiner V3 Catheter Competitive Brochure	SAM	Product Manager
GuideLiner V3 Catheter 5F Product Brochure	SAM	Product Manager
GuideLiner V3 Catheter Case Reports (Brochure and Insert)	SAM	Product Manager
In-Service Presentation	SAM	Product Manager
Deployment Video	SAM	Product Manager
IFU	SAM	Product Manager

TrapLiner Catheter

Item	Location	Contact for More Information
Coronary Demo Model	Received after New Hire Training	Sales/Marketing Coordinator
TrapLiner Catheter Product Brochure	SAM	Product Manager
In-Service Presentation	SAM	Product Manager
Deployment Video	SAM	Product Manager
IFU	SAM	Product Manager

Legal and Regulatory Disclosures

The GuideLiner V3 Catheters are intended to be used in conjunction with guide catheters to access discrete regions of the coronary and/or peripheral vasculature, and to facilitate placement of interventional devices.

The TrapLiner Catheter is intended for use in conjunction with guide catheters to access discrete regions of the coronary and/or peripheral vasculature, to facilitate placement of interventional devices, and to facilitate the exchange of an interventional device while maintaining the position of a guidewire within the vasculature.

The GuideLiner V3 Catheters are contraindicated in vessels less than 2.5mm in diameter and in vessels in the neurovasculature or the venous system.

The TrapLiner Catheter is contraindicated in vessels smaller than its tip outer diameter and in vessels in the neurovasculature.

Please see the Instructions for Use for a complete listing of the indications, contraindications, warnings and precautions.

Teleflex, the Teleflex logo, Guardian, GuideLiner and TrapLiner are trademarks or registered trademarks of Teleflex incorporated or its affiliates, in the U.S. and/or other countries. Other names may be trademarks of their respective owners.

Information in this materials is not a substitute for the product Instructions for Use. Not all products may be available in all countries. Please contact your local representative.

©2019 Teleflex Incorporated. All rights reserved. MCI-2019-0561-EN-SG 08 19

Distributed by:
Teleflex Headquarters International, Ireland · Teleflex Medical Europe Ltd. · IDA Business & Technology Park Dublin Road · Athlone · Co Westmeath · Tel. +353(0)9 06 46 08 00 · Fax +353(0)14 37 07 73 · orders.intl@teleflex.com
Singapore · Teleflex Medical Asia Pte. Ltd. · Tel +65 6439 3000 · TMACSGroup@teleflex.com · www.teleflex.com